



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هدى بشير محمد طلبه

العنوان : قرية صول

الحي / المدينة : اطفح

المحافظة : الجيزه

تاريخ تسجيل الإستماره : 01-10-2026

رقم البطاقة : 26107132101903 رقم التليفون : 01115728779

العمر (تاريخ الميلاد) : 13-07-1961 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هدى بشير محمد طلبة National ID: 26107132101903	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



دولة الكويت
مركز التطوير

الاسم
بشير محمد طه
طرية صول
مركز التطوير - الكويت

٢٠١١.٠٧.١٣ ٢١.٠١٩.٣

LR3572342



رقم الهوية
٢١٠٧٢٣٤٢
اللقب
بشير محمد طه
مركز التطوير
٢٠١١.٠٧.١٣ ٢١.٠١٩.٣



Dms

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٢٠١١.٠٧.١٣ ٢١.٠١٩.٣

MDT FORM

Name:	Hoda محمد هادي	MRN:	10518
Age:	65 Y	ID:	
Menopausal Sta	Post	Phone:	01115728779
NRS SCORE:	ECOG P51	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	new case

Clinical Data:

BX:

ER: 0/8 PR: 0/8 HER2: +2 not ar K167: 70

R

L

Sonomammography:

mammo 24/3/26: at women health It breast UOQ two solid masses , largest (3.9x2 cm) show intraductal extension +multiple axillary LNs (more than 5 at level I and II LT breast IDC ER 0/8 PR 0/8 Her 2 not amplified by SISH clinically: It UOQ mass 4x3 cm + multiple palpable mobile axillary LNs

Mets Workup:

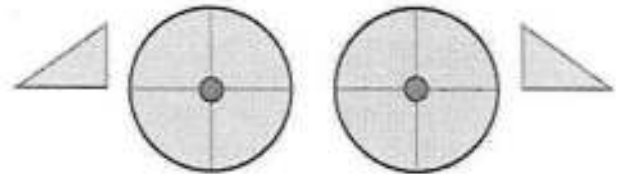
CT CAP Free
Bone SCan Free

History:

Recommendations:

For ECHO- Porta cath - Labs
BRCA test
Keynot 522 Protocol as NACT
Lt MRM , Rt MDE

MDT Outcome & Plan



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	هدى بشير محمد	Case No.	296778
Sex/Age	Female 64 Y	Patient No.	21787
Registered Date	15/6/2026 02:20:58 PM	Referred By Dr.	Interventional Radiology
Validation Date	06/7/2026 08:49:00 AM	Branch	Gustave Roussy
Reporting Date	07/7/2026 09:07:36 AM	Pathology No.	1886/26

Pathology Report

Clinical Data: Right axillary indeterminate lymph node in a patient with pathologically proved triple negative left breast cancer.

Nature of Specimen: Tru-cut biopsy, collected on 15/06/2026, at 12:05 pm.
- Cold ischemia time: 20 seconds.

Gross Examination:

- Two gray-white tissue cores measured 0.5 cm & 0.8 cm long, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of lymphoid tissue formed of small & intermediate sized lymphocytes arranged in follicles with vague germinal centers formation. There is focal dermatopathic changes in the form of macrophages with melanin pigment. No evidence of metastatic deposits. No evidence of malignancy in section examined.

Diagnosis:

Right axillary lymph node, Tru-cut biopsy:

Reactive nodal tissue with dermatopathic changes. No evidence of metastatic deposits in specimen received. No malignancy.

Signature

Dr. Walaa Ibrahim Ghonaim.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mier El Qadema - Cairo, Egypt



هلاي
بشير محمد طلبة
قرية صول
مركز أطفيج - الجيزة

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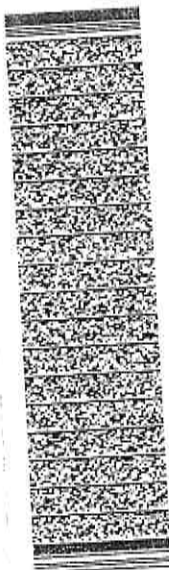
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2012/12/17

۲۰۲۶۱-۳۲۱۰۱۹۰۳
ریبه مغزول

مفتوحة
معلمة
جسك ابراهيم محمد طلبة

البطاقة المالية حتى ٢٠٣٣/٠٢/٢٣



Patient Name	: الأسكنانه / هدى بشير محمد طلبة	Request No.	: 13005771
Patient No.	: 26107132101903	Request Date	: 08/04/2026
Path No.	: 931/26	Verification Date	: 21/04/2026
Sex	: Female	Financial Category	: مستشفى هرمل - جوسسيف روسي
Age	: 64 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast UOQ mass, "BIRADs 5".

Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

Gross Examination:

- Five grey white tissue cores, ranging in size between 1.0 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angiolymphatic invasion: Absent.
- * Necrosis: Focal.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 26/04/2026 09:37:31

Page 1 of 1

Printing By : marwa mohamed youss



Patient Name الأستاذة / هدى بشير محمد طلبه
Patient No. 26107132101903
Path No. 931/26
Sex Female
Age 64 Y
Refereing Doc.

Request No. 13005771
Request Date 08/04/2026
Verification Date 21/04/2026
Financial Category جرسنكف روسي - مستشفى هرمل
Department Out Patient
Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Verified by :

Dr.Duaa Mohamed

Printing Date : 26/04/2026 09:37:35

Page 2 of 3

Printed By : .marwa mohamed yousse



Patient Name : الأستاذ / هدى بشير محمد طلبة
Patient No. : 26107132101903
Path No. : 931/26
Sex : Female
Age : 64 Y
Referring Doc. :

Request No. : 13005771
Request Date : 08/04/2026
Verification Date : 21/04/2026
Financial Category : مستشفى هرمي - جويستاف روسي
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

Immunohistochemistry
[DOCTYPE:IHC]

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast UOQ mass, "BIRADs 5" diagnosed invasive duct carcinoma, grade 3.
Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: (2.6), Her2/CEP17 ratio: (1.26)).
- Ki67%: 70%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
- (Average Her2 copy No.: (2.6), Her2/CEP17 ratio: (1.26)).
- Ki67%: 70%.

* Scoring system: "ER, PR"

Verified by :

Dr. Duaa Mohamed

Printing Date : 26/04/2026 09:37:35

Page 1 of 3

Printed By : marwa mohamed yousse



Patient ID: 93913	Patient Name English: hdi Beshir Mohamed
مرضى ٩٣٩١٣	2026/03/24 10:42:23 AM . Report Date

Women's Health Outreach Program

Name: Hoda Beshir Mohamed

Date: 24/3/2026

Bilateral Full field Digital Mammography

Clinical summary: 65 y old lady coming for left breast lump one week duration .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).
- UOQ show lobulated isodense mass with angular margin another near by irregular shaped hyper dense mass .
- **Ultrasound** demonstrated A large lobulated hypoechoic heterogeneous solid mass lesion measures 3.9x2 cm with anterior and posterior ductal extension seen at 1 o'clock 4 cm from nipple 5 mm from skin with increased vascularity by color Doppler study .
- Another irregular hypoechoic solid mass lesion seen at 12 o'clock (Zone B)4 cm from nipple 1 cm from skin ,1.7 cm away from index mass connected to each other by dilated duct ,it measures 2.4x1.2 cm
- Normal skin and nipple/areolar complex.
- Multiple enlarged pathological axillary L.Ns level I and II (more than 5) some of them shows focal cortical thickening other with lost hilum ,largest measures 1.7x0.8 cm with focal cortical thickening 4.3 mm

Right breast

- Scattered fibro glandular densities (ACR B).
- Retroareolar area is the seat of focal asymmetry .
- No masses, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated no solid or cystic lesions yet prominent retroareolar duct system reaching 2.8 mm with thickened wall and internal isoechoic content likely secretion .
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Left breast UOQ two suspicious solid mass lesion largest show ductal extension with multiple (more than 5) axillary level I and II pathological L.Ns (BIRADS 5)

Right breast : retroareolar prominent ducts with thickened wall likely inflammatory (BIRADS 3).

Recommendations:

For tru cut biopsy and histopathological correlation .

ECHO REPORT

NAME : 3-3-20

DATE: 5 / 8 / 2026

ID NUMBER :

SEX : AGE:

EF	55-75%	PWT	0.6-1.1cm
LVEDD	4.6	AO.R	2.1
LVEDD	2.3	LA	3.6
FS	26%	RV	2.4cm
SWT	0.9	AO.CUSP	0.7-2.3cm(supine)
			1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR:

TAPS: 2.3

Name : Huda Bashir Mohammed Talba

Date : 18-06-2026 07:41

Branch : Dokki

Medical No. : 396856

Ref.Physician Prof : Goustav Roussey Egypt

Whole-Body Bone Scintigraphy Report

Clinical History & Indication: 65-year-old female patient; Left breast carcinoma status post chemotherapy and radiotherapy; evaluated for secondary skeletal metastasis due to generalized bony aches.

Technique

Following the intravenous administration of Tc-99m MDP, anterior and posterior whole-body planar scans were acquired.

Findings

- Uniform, symmetric radiotracer distribution without focal hypermetabolic lesions or fractures across the entire axial and appendicular skeleton.
- Degenerative Joint Changes: Prominent, symmetric radiotracer accumulation localized to both knees and ankles.
- Normal background soft-tissue clearance; symmetric renal excretion with standard physiological pooling in the urinary bladder.

Impression

- **No Scintigraphic Evidence of Osteoblastic Metastatic Disease:** Reassuring scan showing no evidence of asymmetric or multi-focal skeletal metastases.
- Clinical follow-up as per standard post-treatment oncological protocols.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient's Name: Huda Bashir Mohammed Faiba
Date: 2026-05-19
Referred by: Prof.Dr. no doctor
Hospital: Gonslave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Left breast lesion is seen measuring 36 x 34 mm for sono-mammography.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ No pulmonary or hepatic metastatic lesions.

Dr.Sherif Wagdy

Dr.Sherif Wagdy



هلاي
بشير محمد طلبة
قرية صول
مركز أطفيج - الجيزة

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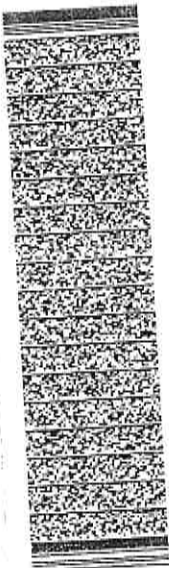
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2012/12/17

۲۰۲۶۱-۳۲۱-۱۹۰۳
ریبه مغزول

مترجمة
مصلحة
الشيخ
جمال إبراهيم محمد طلبة

البطاقة صاريه حتى ٢٠٣٣/٠٢/٢٣



Patient Name	: الأسكنانه / هدى بشير محمد طلبة	Request No.	: 13005771
Patient No.	: 26107132101903	Request Date	: 08/04/2026
Path No.	: 931/26	Verification Date	: 21/04/2026
Sex	: Female	Financial Category	: مستشفى هرمل - جوسسيف روسي
Age	: 64 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast UOQ mass, "BIRADs 5".

Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

Gross Examination:

- Five grey white tissue cores, ranging in size between 1.0 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angiolymphatic invasion: Absent.
- * Necrosis: Focal.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 26/04/2026 09:37:31

Page 1 of 1

Printing By : marwa mohamed youss



Patient Name الأستغانه / هدى بشير محمد طلبه
Patient No. 26107132101903
Path No. 931/26
Sex Female
Age 64 Y
Refereing Doc.

Request No. 13005771
Request Date 08/04/2026
Verification Date 21/04/2026
Financial Category جرسنلف روسي - مستشفى هرمل
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

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- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Verified by :

Dr.Duaa Mohamed

Printing Date : 26/04/2026 09:37:35

Page 2 of 3

Printed By : .marwa mohamed yousse



Patient ID: 93913	Patient Name English: hdi Beshir Mohamed
مرضى ٩٣٩١٣	2026/03/24 10:42:23 AM . Report Date

Women's Health Outreach Program

Name: Hoda Beshir Mohamed

Date: 24/3/2026

Bilateral Full field Digital Mammography

Clinical summary: 65 y old lady coming for left breast lump one week duration .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).
- UOQ show lobulated isodense mass with angular margin another near by irregular shaped hyper dense mass .
- **Ultrasound** demonstrated A large lobulated hypoechoic heterogeneous solid mass lesion measures 3.9x2 cm with anterior and posterior ductal extension seen at 1 o'clock 4 cm from nipple 5 mm from skin with increased vascularity by color Doppler study .
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Impression:

Left breast UOQ two suspicious solid mass lesion largest show ductal extension with multiple (more than 5) axillary level I and II pathological L.Ns (BIRADS 5)

Right breast : retroareolar prominent ducts with thickened wall likely inflammatory (BIRADS 3).

Recommendations:

For tru cut biopsy and histopathological correlation .

ECHO REPORT

NAME : Youssef

DATE: 5 / 8 / 2026

ID NUMBER : /

SEX : / AGE: /

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVEDS	2.3	2-4cm	LA	3.6	2-4cm
FS	26%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GI

SWMA No RCM

LA and Ao Root Dimensions Normal

Mitral Valve SL ± mild MR

Aortic Valve SL ± mild AR

Tricuspid Valve mild TR

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

TAPS: 2.3

DR:

[Signature]

Name : Huda Bashir Mohammed Talba

Date : 18-06-2026 07:41

Branch : Dokki

Medical No. : 396856

Ref.Physician Prof : Goustav Roussey Egypt

Whole-Body Bone Scintigraphy Report

Clinical History & Indication: 65-year-old female patient; Left breast carcinoma status post chemotherapy and radiotherapy; evaluated for secondary skeletal metastasis due to generalized bony aches.

Technique

Following the intravenous administration of Tc-99m MDP, anterior and posterior whole-body planar scans were acquired.

Findings

- Uniform, symmetric radiotracer distribution without focal hypermetabolic lesions or fractures across the entire axial and appendicular skeleton.
- Degenerative Joint Changes: Prominent, symmetric radiotracer accumulation localized to both knees and ankles.
- Normal background soft-tissue clearance; symmetric renal excretion with standard physiological pooling in the urinary bladder.

Impression

- **No Scintigraphic Evidence of Osteoblastic Metastatic Disease:** Reassuring scan showing no evidence of asymmetric or multi-focal skeletal metastases.
- Clinical follow-up as per standard post-treatment oncological protocols.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient's Name: Huda Bashir Mohammed Faiba
Date: 2026-05-19
Referred by: Prof.Dr. no doctor
Hospital: Gonslave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Left breast lesion is seen measuring 36 x 34 mm for sono-mammography.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ No pulmonary or hepatic metastatic lesions.

Dr.Sherif Wagdy

Dr.Sherif Wagdy



هلاي
بشير محمد طلبة
قرية صول
مركز أطفيج - الجيزة

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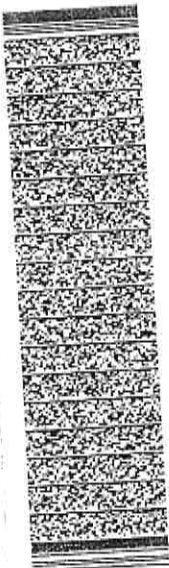
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ریبه مغزل

مترجمة
مسلمة
الشيخ
جمال إبراهيم محمد طلبة

البطاقة منارية حتى ٢٠٣٣/٠٢/٢٣



Patient Name	: الأسكنانه / هدى بشير محمد طلبة	Request No.	: 13005771
Patient No.	: 26107132101903	Request Date	: 08/04/2026
Path No.	: 931/26	Verification Date	: 21/04/2026
Sex	: Female	Financial Category	: مستشفى هرمي - جوسناف روسي
Age	: 64 Y	Department	: Out Patient
Referring Doc		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast UOQ mass, "BIRADs 5".

Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

Gross Examination:

- Five grey white tissue cores, ranging in size between 1.0 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angiolymphatic invasion: Absent.
- * Necrosis: Focal.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 26/04/2026 09:37:31

Page 1 of 1

Printing By : marwa mohamed youss



Patient Name الأستاذة / هدى بشير محمد طلبة
Patient No. 26107132101903
Path No. 931/26
Sex Female
Age 64 Y
Refereing Doc.

Request No. 13005771
Request Date 08/04/2026
Verification Date 21/04/2026
Financial Category جرسنلف روسي - مستشفى هرمل
Department Out Patient

Registered by marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Verified by :

Dr.Duaa Mohamed

Printing Date : 26/04/2026 09:37:35

Page 2 of 3

Printed By : marwa mohamed yousse



Patient Name : الأستاذ / هدى بشير محمد طلبة
Patient No. : 26107132101903
Path No. : 931/26
Sex : Female
Age : 64 Y
Referring Doc. :

Request No. : 13005771
Request Date : 08/04/2026
Verification Date : 21/04/2026
Financial Category : مستشفى هرمي - جويستاف روسي
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

Immunohistochemistry
[DOCTYPE:IHC]

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast UOQ mass, "BIRADs 5" diagnosed invasive duct carcinoma, grade 3.
Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
- (Average Her2 copy No.: (2.6), Her2/CEP17 ratio: (1.26)).
- Ki67%: 70%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
- (Average Her2 copy No.: (2.6), Her2/CEP17 ratio: (1.26)).
- Ki67%: 70%.

* Scoring system: "ER, PR"

Verified by :

Dr. Duaa Mohamed

Printing Date : 26/04/2026 09:37:35

Page 1 of 3

Printed By : marwa mohamed yousse



Patient ID: 93913	Patient Name English: hdi Beshir Mohamed
مرضى ٩٣٩١٣	2026/03/24 10:42:23 AM . Report Date

Women's Health Outreach Program

Name: Hoda Beshir Mohamed

Date: 24/3/2026

Bilateral Full field Digital Mammography

Clinical summary: 65 y old lady coming for left breast lump one week duration .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).
- UOQ show lobulated isodense mass with angular margin another near by irregular shaped hyper dense mass .
- **Ultrasound** demonstrated A large lobulated hypoechoic heterogeneous solid mass lesion measures 3.9x2 cm with anterior and posterior ductal extension seen at 1 o'clock 4 cm from nipple 5 mm from skin with increased vascularity by color Doppler study .
- Another irregular hypoechoic solid mass lesion seen at 12 o'clock (Zone B)4 cm from nipple 1 cm from skin ,1.7 cm away from index mass connected to each other by dilated duct ,it measures 2.4x1.2 cm
- Normal skin and nipple/areolar complex.
- Multiple enlarged pathological axillary L.Ns level I and II (more than 5) some of them shows focal cortical thickening other with lost hilum ,largest measures 1.7x0.8 cm with focal cortical thickening 4.3 mm

Right breast

- Scattered fibro glandular densities (ACR B).
- Retroareolar area is the seat of focal asymmetry .
- No masses, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated no solid or cystic lesions yet prominent retroareolar duct system reaching 2.8 mm with thickened wall and internal isoechoic content likely secretion .
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Left breast UOQ two suspicious solid mass lesion largest show ductal extension with multiple (more than 5) axillary level I and II pathological L.Ns (BIRADS 5)

Right breast : retroareolar prominent ducts with thickened wall likely inflammatory (BIRADS 3).

Recommendations:

For tru cut biopsy and histopathological correlation .

ECHO REPORT

NAME : 3-3-20

DATE: 5 / 8 / 2026

ID NUMBER :

SEX : AGE:

EF	55-75%	PWT	0.6-1.1cm
LVEDD	4.6	AO.R	2.1
LVEDD	2.3	LA	3.6
FS	26%	RV	2.4cm
SWT	0.9	AO.CUSP	0.7-2.3cm(supine)
			1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR:

TAPS: 2.3

Name : Huda Bashir Mohammed Talba

Date : 18-06-2026 07:41

Branch : Dokki

Medical No. : 396856

Ref.Physician Prof : Goustav Roussey Egypt

Whole-Body Bone Scintigraphy Report

Clinical History & Indication: 65-year-old female patient; Left breast carcinoma status post chemotherapy and radiotherapy; evaluated for secondary skeletal metastasis due to generalized bony aches.

Technique

Following the intravenous administration of Tc-99m MDP, anterior and posterior whole-body planar scans were acquired.

Findings

- Uniform, symmetric radiotracer distribution without focal hypermetabolic lesions or fractures across the entire axial and appendicular skeleton.
- Degenerative Joint Changes: Prominent, symmetric radiotracer accumulation localized to both knees and ankles.
- Normal background soft-tissue clearance; symmetric renal excretion with standard physiological pooling in the urinary bladder.

Impression

- **No Scintigraphic Evidence of Osteoblastic Metastatic Disease:** Reassuring scan showing no evidence of asymmetric or multi-focal skeletal metastases.
- Clinical follow-up as per standard post-treatment oncological protocols.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient's Name: Huda Bashir Mohammed Faiba
Date: 2026-05-19
Referred by: Prof.Dr. no doctor
Hospital: Gonslave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- o Left breast lesion is seen measuring 36 x 34 mm for sono-mammography.
- o No pulmonary nodules, masses or cavitary lesions.
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o No normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:

❖ No pulmonary or hepatic metastatic lesions.

Dr.Sherif Wagdy

Dr.Sherif Wagdy

Patient Name	د. محمد محمد طلبة	Case No.	296778
Sex/Age	Female 64 Y	Patient No.	21787
Registered Date	15/6/2026 02:20:58 PM	Referred By Dr.	Interventional Radiology
Validation Date	06/7/2026 08:49:00 AM	Branch	Gustave Roussy
Reporting Date	24/9/2026 09:18:09 AM	Pathology No.	1886/26

Pathology Report

Clinical Data: Right axillary indeterminate lymph node in a patient with pathologically proved triple negative left breast cancer.

Nature of Specimen: Tru-cut biopsy, collected on 15/06/2026, at 12:05 pm.
- Cold ischemia time: 20 seconds.

Gross Examination:
- Two gray-white tissue cores measured 0.5 cm & 0.8 cm long, totally submitted.

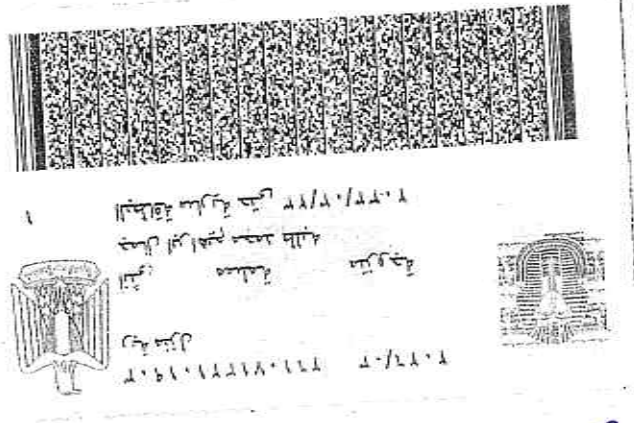
Microscopic Examination:
- Histopathological examination reveals cores of lymphoid tissue formed of small & intermediate sized lymphocytes arranged in follicles with vague germinal centers formation. There is focal dermatopathic changes in the form of macrophages with melanin pigment. No evidence of metastatic deposits. No evidence of malignancy in section examined.

Diagnosis:
Right axillary lymph node, Tru-cut biopsy:
Reactive nodal tissue with dermatopathic changes. No evidence of metastatic deposits in specimen received. No malignancy.

Signature
Dr. Walaa Ibrahim Ghonaim.

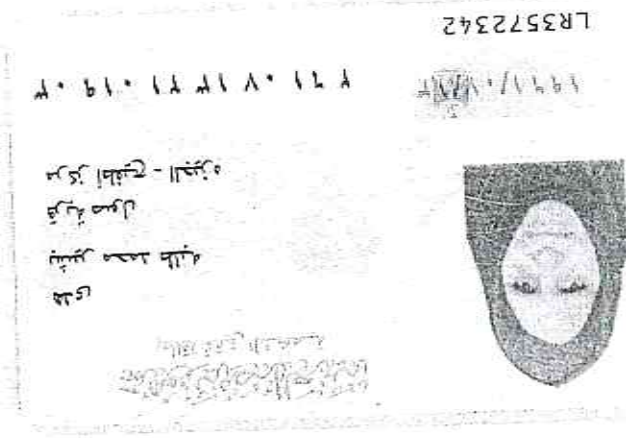


مركز روستي الدولي للبحوث المرضية - مصر - القاهرة
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Al-Her El Qadima - Cairo, Egypt



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٢٠١٦/٠٣/٠٣





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عزقة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر

Printing Date : 26/04/2026 09:37:31 Page 1 of 1 Printing By : marwa mohamed youss

Verified by :
Dr.Duaa Mohamed

Signature
Dr. Walaa Ibrahim Ghoneim.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angiolymphatic invasion: Absent.
* Necrosis: Focal.

Gross Examination:
- Five grey white tissue cores, ranging in size between 1.0 cm & 1.5 cm long, totally submitted.

Clinical Data: Left breast UOQ mass, "BIRADS 5".
Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

Pathology Report

Patient Name	الأستاذة / هادي بشير محمد طلبة
Patient No.	26107132101903
Path No.	931/26
Sex	Female
Age	64 Y
Referring Doc :	Registered by : marwa mohamed youss
Request No.	13005771
Request Date	08/04/2026
Verification Date	21/04/2026
Financial Category	مستشفى هرمي - مستشفى روسي - جوستاف روسي
Department	Out Patient

ROUSSY
JUSTAVE
INTERNATIONAL
EGYPT



Verified by :
 Dr.Duaa Mohamed

4. Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4. (Recounting give the same result).

4. Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).

4. Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).

4. Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

4. Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

The ratio is calculated and the results is interpreted as:

ISH groups:

intense > 10% of tumor cells.

10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < the invasive tumor cells or complete and circumferential membrane staining that is intense and within < within > 10% of IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining * Scoring system: "Her2"

intermediate; and 3, strong). Total score from 0 to 8.

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

IMMUNOHISTOCHEMISTRY REPORT

Registered by	marwa mohamed yousse
Reffering Doc.	
Age	64 Y
Sex	Female
Path No.	931/26
Patient No.	26107132101903
Patient Name	الانسانه / هادي بشير محمد طه
Request No.	13005771
Request Date	08/04/2026
Verification Date	21/04/2026
Financial Category	المرضى - مستشفى جوفه
Department	Out Patient

Patient ID: 93913	Patient Name English: hdi Beshir Mohamed
Age: 65	2026/03/24 10:42:23 AM : Report Date

Women's Health Outreach Program

Name: Hoda Beshir Mohamed

Date: 24/3/2026

Bilateral Full field Digital Mammography

Clinical summary: 65 y old lady coming for left breast lump one week duration .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).

- UOQ show lobulated isodense mass with angular margin another near by

- irregular shaped hyper dense mass .

- Ultrasound** demonstrated A large lobulated hypoechoic heterogeneous solid

- mass lesion measures 3.9x2 cm with anterior and posterior ductal extension

- seen at 1 o'clock 4 cm from nipple 5 mm from skin with increased vascularity

- by color Doppler study .

- Another irregular hypoechoic solid mass lesion seen at 12 o'clock (Zone B)4 cm

- from nipple 1 cm from skin ,1.7 cm away from index mass connected to each

- other by dilated duct ,it measures 2.4x1.2 cm

- Normal skin and nipple/areolar complex.

- Multiple enlarged pathological axillary L.Ns level I and II (more than 5)

- some of them shows focal cortical thickening other with lost hilum

- ,largest measures 1.7x0.8 cm with focal cortical thickening 4.3 mm

Right breast

- Scattered fibro glandular densities (ACR B).

- Retroareolar area is the seat of focal asymmetry .

- No masses, distortion or suspicious clusters of micro calcifications.

- Ultrasound** demonstrated no solid or cystic lesions yet prominent retroareolar

- duct system reaching 2.8 mm with thickened wall and internal isoechoic

- content likely secretion .

- Normal skin and nipple/areolar complex.

- Preserved axillary LNs.

Impression:

Left breast UOQ two suspicious solid mass lesion largest show ductal extension

with multiple (more than 5) axillary level I and II pathological L.Ns (BIRADS 5)

Right breast : retroareolar prominent ducts with thickened wall likely

inflammatory (BIRADS 3).

Recommendations:

For tru cut biopsy and histopathological correlation .

ECHO REPORT

NAME: *Dr. [Signature]*

ID NUMBER: /

SEX: AGF:

DATE: 5 / 8 / 2026

EF	62%	55-75%	PWT	0.6-1.1cm	AO.CUSP	1.5-2.6cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.4cm	LA	2-4cm
LVEDSD	2.3	2.4cm	LA		RV	0.7-2.3cm(supine)
FS	26%	25-45%	RV			
SWT	0.9	0.6-1.1cm	AO.CUSP			

LV Dimensions *Normal*

LV Wall Thickness *Normal*

LV Diastolic Function *LT*

SWMA *No RWM*

LA and Ao Root Dimensions *Normal*

Mitral Valve *SL E mild MR*

Aortic Valve *SL E mild AR*

Tricuspid Valve *Mild TR*

Pulmonary Valve

Right Side and Systolic Function

APSE: 2.3

DR:

[Signature]

Date : 18-06-2026 07:41

Medical No. : 396856

Ref. Physician Prof : Goustav Roussey Egypt

Branch : Dokki

Name : Huda Bashir Mohammed Talba

Whole-Body Bone Scintigraphy Report

Clinical History & Indication: 65-year-old female patient; Left breast carcinoma status post chemotherapy and radiotherapy; evaluated for secondary skeletal metastasis due to generalized bony aches.

Technique
Following the intravenous administration of Tc-99m MDP, anterior and posterior whole-body planar scans were acquired.

Findings
• Uniform, symmetric radiotracer distribution without focal hypermetabolic lesions or fractures across the entire axial and appendicular skeleton.
• Degenerative Joint Changes: Prominent, symmetric radiotracer accumulation localized to both knees and ankles.
• Normal background soft-tissue clearance; symmetric renal excretion with standard physiological pooling in the urinary bladder.

Impression
• No Scintigraphic Evidence of Osteoblastic Metastatic Disease: Reassuring scan showing no evidence of asymmetric or multi-focal skeletal metastases.
• Clinical follow-up as per standard post-treatment oncological protocols.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Patient's Name: Huda Bashir Mohammed Talba
Date: 2026-05-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Left breast lesion is seen measuring 36 x 34 mm for sono-mammography.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ No pulmonary or hepatic metastatic lesions.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Patient Name	د. محمد محمد علي	Case No.	296778
Sex/Age	Female 64 Y	Patient No.	21787
Registered Date	15/6/2026 02:20:58 PM	Referred By Dr.	Interventional Radiology
Validation Date	06/7/2026 08:49:00 AM	Branch	Gustave Roussy
Reporting Date	24/9/2026 09:18:09 AM	Pathology No.	1886/26

Pathology Report

Clinical Data: Right axillary indeterminate lymph node in a patient with pathologically proved triple negative left breast cancer.

Nature of Specimen: Tru-cut biopsy, collected on 15/06/2026, at 12:05 pm.
- Cold ischemia time: 20 seconds.

Gross Examination: - Two gray-white tissue cores measured 0.5 cm & 0.8 cm long, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of lymphoid tissue formed of small & intermediate sized lymphocytes arranged in follicles with vague germinal centers formation. There is focal dermatopathic changes in the form of macrophages with melanin pigment. No evidence of metastatic deposits. No evidence of malignancy in section examined.

Diagnosis:

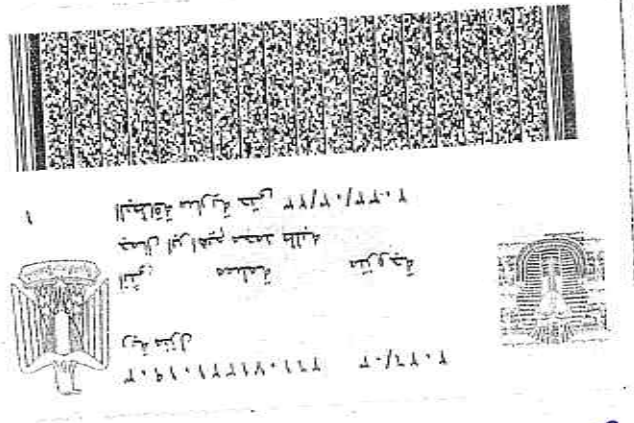
Right axillary lymph node, Tru-cut biopsy:

Reactive nodal tissue with dermatopathic changes. No evidence of metastatic deposits in specimen received. No malignancy.

Signature
Dr. Walaa Ibrahim Ghonaim.

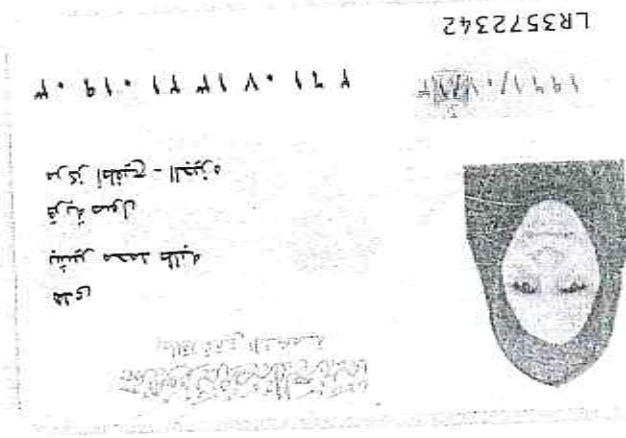


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٢٠١٦/٠٣/٠٣





Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt
 غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر

Printing Date : 26/04/2026 09:37:31 Page 1 of 1 Printing By : marwa mohamed youss

Verified by :
 Dr.Duaa Mohamed

Signature
 Dr. Walaa Ibrahim Ghoneim.

Diagnosis:
 Left breast mass, Tru-cut biopsy:
 - Invasive duct carcinoma, Grade 3.
 - For further IHC (ER, PR, Her2, Ki67).

Microscopic Examination:
 * Histological type: Invasive duct carcinoma.
 * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
 * Ductal carcinoma in situ: Absent.
 * Perineural invasion: Absent.
 * Angiolymphatic invasion: Absent.
 * Necrosis: Focal.

Gross Examination:
 - Five grey white tissue cores, ranging in size between 1.0 cm & 1.5 cm long, totally submitted.

Clinical Data: Left breast UOQ mass, "BIRADS 5".
 Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

Pathology Report

Patient Name	الأستاذة / هادي بشير محمد طلبة
Patient No.	26107132101903
Path No.	931/26
Sex	Female
Age	64 Y
Referring Doc :	Registered by : marwa mohamed youss
Request No.	13005771
Request Date	08/04/2026
Verification Date	21/04/2026
Financial Category	مستشفى هرمي - مستشفى روسي جوستاف
Department	Out Patient

ROUSSY
 JUSTAVE
 INTERNATIONAL
 EGYPT



Verified by :
 Dr.Duaa Mohamed

4. Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4. (Recounting give the same result).

4. Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).

4. Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).

4. Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

4. Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

The ratio is calculated and the results is interpreted as:

ISH groups:

intense > 10% of tumor cells.

10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < the invasive tumor cells or complete and circumferential membrane staining that is intense and within < within > 10% of IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining

* Scoring system: "Her2"

intermediate; and 3, strong). Total score from 0 to 8.

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

IMMUNOHISTOCHEMISTRY REPORT

Registered by	marwa mohamed yousse
Referred by	marwa mohamed yousse
Age	64 Y
Sex	Female
Path No.	931/26
Patient No.	26107132101903
Patient Name	الانسانه / هادي بنه محمد طاهر
Request No.	13005771
Request Date	08/04/2026
Verification Date	21/04/2026
Financial Category	المرضى - مستشفى جوفه
Department	Out Patient



Dr.Duaa Mohamed

Verified by :

* Scoring system: "ER, PR"

- ER: Negative (score 0/8).
 - PR: Negative (score 0/8).
 - Her2: Equivocal (score 2+).
 - SISH: Not amplified, Group V.
 (Average Her2 copy No.: (2.6), Her2/CEP17 ratio: (1.26)).
 - Ki67%: 70%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

* IPT (All controls showed appropriate reactivity):
 - ER: Negative nuclear reaction in tumor cells (score 0/8).
 - PR: Negative nuclear reaction in tumor cells (score 0/8).
 - Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
 - SISH: Not amplified.
 (Average Her2 copy No.: (2.6), Her2/CEP17 ratio: (1.26)).
 - Ki67%: 70%.

Results:

Technique:
 - Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Clinical Data: Left breast UOQ mass, "BIRADS 5" diagnosed invasive duct carcinoma, grade 3.
 Nature of Specimen: Tru-cut biopsy, collected on 8/4/2026, at 9:45 am, Cold ischemia time: 20 seconds.

IMMUNOHISTOCHEMISTRY REPORT

Request No.	13005771	Request Date	08/04/2026	Verification Date	21/04/2026	Financial Category	جولف روي - مستشفى هرمي	Department	Out Patient	IHC No.	:	Registered by	marwa mohamed yousse
Patient Name	المنصور / هادي بنير محمد طلبة	Patient No.	26107132101903	Path No.	931/26	Sex	Female	Age	64 Y	Reffering Doc.	:		

Patient ID: 93913	Patient Name English: hdi Beshir Mohamed
مرحباً أ. هدى ب. هادي	Age: 65
2026/03/24 10:42:23 AM	Report Date

Women's Health Outreach Program

Name: Hoda Beshir Mohamed

Date: 24/3/2026

Bilateral Full field Digital Mammography

Clinical summary: 65 y old lady coming for left breast lump one week duration .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).

- UOQ show lobulated isodense mass with angular margin another near by

- irregular shaped hyper dense mass .

- Ultrasound demonstrated A large lobulated hypoechoic heterogeneous solid

- mass lesion measures 3.9x2 cm with anterior and posterior ductal extension

- seen at 1 o'clock 4 cm from nipple 5 mm from skin with increased vascularity

- by color Doppler study .

- Another irregular hypoechoic solid mass lesion seen at 12 o'clock (Zone B)4 cm

- from nipple 1 cm from skin ,1.7 cm away from index mass connected to each

- other by dilated duct ,it measures 2.4x1.2 cm

- Normal skin and nipple/areolar complex.

- Multiple enlarged pathological axillary L.Ns level I and II (more than 5)

- some of them shows focal cortical thickening other with lost hilum

- largest measures 1.7x0.8 cm with focal cortical thickening 4.3 mm

Right breast

- Scattered fibro glandular densities (ACR B).

- Retroareolar area is the seat of focal asymmetry .

- No masses, distortion or suspicious clusters of micro calcifications.

- Ultrasound demonstrated no solid or cystic lesions yet prominent retroareolar

- duct system reaching 2.8 mm with thickened wall and internal isoechoic

- content likely secretion .

- Normal skin and nipple/areolar complex.

- Preserved axillary LNs.

Impression:

Left breast UOQ two suspicious solid mass lesion largest show ductal extension

with multiple (more than 5) axillary level I and II pathological L.Ns (BIRADS 5)

Right breast : retroareolar prominent ducts with thickened wall likely

inflammatory (BIRADS 3).

Recommendations:

For tru cut biopsy and histopathological correlation .

NAME: _____

ID NUMBER:

SEX

FDW

DATE:

9702/8/5

LV Dimensions	LV Wall Thickness
Normal	Normal

LV Diastolic Function

SWMA No. RWMA

LA and Ao Root Dimensions

Aortic Valve

Tricuspid Val

Pulmonary Valve

Right Side and Systolic Function

2.3. $\mathcal{A}^{\text{psf}}$

DR

2015

Name : Huda Bashir Mohammed Talba

Branch : Dokki

Ref. Physician Prof : Goustav Roussey Egypt

Date : 18-06-2026 07:41

Medical No. : 396856

Whole-Body Bone Scintigraphy Report

Clinical History & Indication: 65-year-old female patient; Left breast carcinoma status post chemotherapy and radiotherapy; evaluated for secondary skeletal metastasis due to generalized bony aches.

Technique

Following the intravenous administration of Tc-99m MDP, anterior and posterior whole-body planar scans were acquired.

Findings

- Uniform, symmetric radiotracer distribution without focal hypermetabolic lesions or fractures across the entire axial and appendicular skeleton.
- Degenerative Joint Changes: Prominent, symmetric radiotracer accumulation localized to both knees and ankles.
- Normal background soft-tissue clearance; symmetric renal excretion with standard physiological pooling in the urinary bladder.

Impression

- No Scintigraphic Evidence of Osteoblastic Metastatic Disease: Reassuring scan showing no evidence of asymmetric or multi-focal skeletal metastases.
- Clinical follow-up as per standard post-treatment oncological protocols.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Patient's Name: Huda Bashir Mohammed Talba
Date: 2026-05-19
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Left breast lesion is seen measuring 36 x 34 mm for sono-mammography.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ No pulmonary or hepatic metastatic lesions.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

		جمهورية مصر العربية لدعم صحة المرأة المصرية		08/05/2026
MDT FORM				
Name:	Hoda محمد طلبة	MRN:	10518	
Age:	65 Y	ID:	26107132101903	
Menopausal Sta	Post	Phone:	01115728779	
NRS SCORE:	ECOG PS1	Marital Status:		
BMI:		Family History:		
CO Morbidity:		Reason Of MDT:	new case	

Clinical Data:

BX: **R** **L**

ER: 0/8 **PR:** 0/8 **HER2:** +2 not ar **KI67:** 70

Sonomammography:

mammo 24/3/26: at women health lt breast UOQ two solid masses , largest (3.9x2 cm) show intraductal extension +multiple axillary LNs (more than 5 at level I and II LT breast IDC ER 0/8 PR 0/8 Her 2 not amplified by SISH clinically: lt UOQ mass 4x3 cm + multiple palpable mobile axillary LNs

Mets Workup:

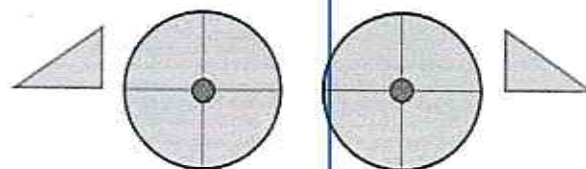
CT CAP Free
 Bone SCan Free

History:**Recommendations:**

For ECHO- Porta cath - Labs
 BRCA test
 Keynote 522 Protocol as NACT
 Lt MRM , Rt MDE

MDT Outcome & Plan

cT2N3 Multifocal with intraductal extension:
 For ECHO- Porta cath - Labs
 BRCA test
 Keynote 522 Protocol as NACT AC x4-- Taxol/Carbo 12 wks all plus
 Keytruda/21days
 Lt MRM , Rt MDE

**T N M****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. تدي حسن	د. محمد لبرلوي				